

Clinical Summary Template

The Children's System of Care will utilize the Clinical Summary Template to assist in the determination of an appropriate intensity of service for any of the following reasons:

1. Linkage with CSOC Care Management Services
2. A supplemental document for Care Management-linked youth, who are currently receiving community-based therapeutic services, and are being referred for OOH treatment.
3. Potential linkage to other CSOC services

Please Note #1: If a youth is participating in the Community Care Waiver through the Division of Developmental Disabilities, the youth is not eligible for CSOC services. Please do not use this document.

Please Note #2: If a youth is in need of Substance Use Treatment services, please refer to the PerformCare Website, at **www.performcarenj.org**, for instructions on how to access services. Please do not use this document.

The Clinical Summary Template is to be completed by an independently licensed clinician (i.e. LCSW, LPC, MD, Ph.D., Psy.D) who is currently providing treatment services to the youth and is informed about the youth's current strengths and needs. All fields are mandatory and must be typed. Download the form to complete, then fax to **1-877-736-9166**.

If you need additional information regarding the clinical summary template, or referring a child for CSOC services, please contact PerformCare at **1-877-652-7624** for assistance.

Clinical Summary Template

Date:	Assessor Name:	Credentials:
Assessor's Agency:		Phone Number:
Email Address:		
Mailing Address:		

Youth's Name:		DOB:
Gender:	Race/Ethnicity:	CYBER ID#
<i>If the youth has not had services through the Children's System Of Care, please instruct the legal guardian to call 877-652-7624 to register the youth.</i>		

Parent/Legal Guardian's Name:		
Guardianship Status (i.e. DCP&P Custody or Guardianship):		
Address:		
City:	State:	Zip:
Primary Phone:		Secondary Phone:
Youth's Current Address (if different from above):		
Primary language spoken in the home:		

Reason for Submission of the Clinical Summary Template:

(i.e. presenting needs, risk of out of home placement, multisystem involvement, current functional status)

Current Status/Treatment & Youth System Involvement:

Behavioral Health (include Outpatient, Intensive In-Home, Partial Hospitalization/ Partial Care Programs, Out of Home, Inpatient Hospitalizations):

Clinical Summary Template

Substance Use (include type of substance, pattern of use, age of onset, types of treatment - Outpatient, Intensive Outpatient, Partial Hospitalization Programs, Short Term Residential, Long Term Residential, Detoxification, Inpatient hospitalizations):		
Physical Health/ Medical (include active Issues):		
Child Welfare/DCP&P (include history of abuse or neglect and descriptions including duration, perpetrator; services including in home services, resource/foster home, therapeutic treatment home):		
DD Eligibility Status (include describing any functional challenges or limitations):		
Juvenile Justice (include case pending, FCIU, probation, detention, day program, DAP, incarceration, parole):		
Specialty Needs (include fire setting, problematic sexual behavior, cruelty to animals, and assaultive behaviors, which has occurred in the past 2 year period – also include details in regards to the severity and frequency of these behaviors and timeline with most recent occurrence):		
School (include type of school placement, i.e. regular education, special education, in district/out of district, home instruction):		
Contact information for DCP&P case worker and supervisors, if involved:		
DCP&P Case Worker:	Phone:	
DCP&P Supervisor:	Phone:	
Current court orders?	☐ Yes	☐ No
Specify:		

Clinical Summary Template

Current Presenting Behavioral Symptoms:

Youth Behavioral/Emotional Needs (please check any behaviors that are relevant to the treatment needs of the youth and/or exhibited in the last 30 days):

☐	Psychosis (Hallucinations, delusional thoughts, bizarre, odd behaviors, speech, and thoughts)	☐	Anxiety (Social anxiety, generalized anxiety, panic symptoms)	☐	Anger Control (The youth's ability to manage their anger)
☐	Impulsivity/Attention (Challenges with impulse control)	☐	Oppositional Behavior (disrespectful, argumentative behaviors, difficulty with accepting rules from authority figures)	☐	Conduct (Antisocial behaviors including stealing, vandalism, cruelty to animals, assaultive behaviors)
☐	Depression (Irritable or depressed mood, isolative, withdrawn behaviors, thoughts of hopelessness, sleep and appetite changes, loss of motivation)	☐	Exposure to Implicit Trauma (Implicit trauma refers to experiences and historical events which may not result in specific memories or overt reactive behaviors, but may contribute to current behavioral/ emotional symptoms) ex. Adoption, loss of a family member	☐	Exposure to Explicit Trauma (Explicit trauma refers to traumatic experiences which directly correlate with post-traumatic emotional and behavioral symptoms) ex. Sexual molestation
☐	Problematic Technology Use (The impact of the technology use on the youth's daily functioning including their ability to maintain relationships, complete school work etc.)	☐	Gambling (Youth's involvement with all forms of gambling, legal and illegal)	☐	Other:

Detailed description of all checked behaviors/symptoms; please include any current presenting symptoms and any history of these symptoms:

Clinical Summary Template

Youth Risk Behaviors (please check any issues that are relevant to the treatment needs of the youth and/or exhibited in the last 30 days):

☐	Suicide Risk (This includes suicidal thoughts, plans, and behaviors)	☐	Danger to Others (This includes actual and threatened violence)
☐	Flight Risk (This includes any planned or unplanned wandering, impulsive running; consider age of the youth, frequency, duration of escape episodes, timing, and context)	☐	Problematic Sexual Behaviors (This includes any sexually aggressive behavior where an older youth takes advantage of a younger youth) (Specialty Evaluation may be indicated if youth is being referred for OOH treatment)
☐	Other Self Harm (Other high risk behaviors which impacts personal safety and increases the risk of personal injury that is not considered suicidal behavior or intentional self-injurious behavior)	☐	Substance Use (This refers to any use of tobacco, alcohol, or illegal drugs) (Specialty Evaluation may be indicated if youth is being referred for OOH treatment)
☐	Self-Injurious Behaviors (Any intentional self-harming behaviors that does not have suicidal intent)	☐	Judgment (This refers to the youth's decision-making ability)
☐	Legal/ Juvenile Justice (This includes any behavior which a youth exhibits that results in involvement with the legal system)	☐	Fire Setting (This refers to when youth intentionally start fires) (Specialty Evaluation may be indicated if youth is being referred for OOH treatment)
☐	Other:		

Detailed description of all checked risk behaviors; please include any current presenting risk behaviors and any history of these risk behaviors:

Clinical Summary Template

Youth Strengths (please check any issues that are relevant to the treatment needs of the youth and/or exhibited in the last 30 days):

☐	Family Strengths - Ability to support the youth's overall progress and development	☐	Relationship Stability - Stability of relationships with friends and family
☐	Talents / Interests which the youth exhibits	☐	Community Involvement - The quality of the youth's connection to their community
☐	Youth's involvement with care and treatment planning	☐	Optimism - Youth's personal sense of optimism
☐	Self-Expression - Youth's ability to express his/her thoughts and feelings	☐	Spirituality - Youth's involvement with spiritual or religious beliefs and practices and activities
☐	Wellness Behaviors - Indicators that the youth exhibits health-promoting behaviors and makes good lifestyle choices	☐	Resiliency - The youth's innate ability to enjoy positive life experiences and manage negative life experiences
☐	Other:		

Detailed description of all checked strengths:

Clinical Summary Template

Life Domain Functioning (please check any issues that are relevant to the treatment needs of the youth and/or exhibited in the last 30 days):

☐	Living Environment (This refers to the youth's functioning in the current living arrangement)	☐	Cultural Stress (This refers to experiences and feelings of discomfort related to real or perceived conflict between an individual's own cultural identity and the predominant culture in which he/she lives in. This includes language barriers, age, gender, ethnicity, physical disability etc.)
☐	Interpersonal Functioning (This refers to the youth's interpersonal skills involving his relationships with peers and non-related adults)	☐	Attachment (This refers to the youth's development of physical and emotional bonding and boundaries with others, specifically within the context of the youth's significant parental or caregiver relationships)
☐	Medical Needs (This includes both acute and chronic medical conditions)	☐	Sleep (This refers to the youth's ability to fall asleep, stay asleep, and wake up on time in the morning)
☐	Eating (This refers to any potential concerns in regards to the youth's food intake, such as overeating, undereating, unusual eating disturbances, and eating disturbances related to distorted body image cognitions)	☐	Sexual Health (This refers to the youth's physical, emotional, mental, and social wellbeing in relation to sexuality)
☐	School Behavior (This refers to any disruptive behavior which the youth exhibits in a school or day care setting)	☐	School Attendance (This refers to how consistently the youth attends school)
☐	Academic Achievement (This refers to the youth's grades and test scores)	☐	Bullied by Others (This refers to the degree to which a youth has been bullied or is being bullied by others)
☐	Learning Disability (This refers to any innate difficulty in a specific academic subject which could require additional educational supports)	☐	Developmental Delay/ Intellectual Disability (Autism, Cerebral Palsy, Down Syndrome, and other Genetic Disorders are rated here)
☐	Educational Agency Involvement (This refers to the School's ability to address youth's educational and behavioral health needs; and the quality of the school's relationship with the youth and family)	☐	Other:

Detailed description of all checked life domain functioning needs; please include any current presenting life domain functioning needs and any history of these life domain functioning needs:

Clinical Summary Template

Caregiver Information/Needs:

Caregiver Name:	Caregiver Relationship to Youth:
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Caregiver strengths and needs (check all that apply):

☐	Physical Disability	☐	Substance Use	☐	Safety of Immediate Living Environment
☐	Mental Health	☐	Developmental Disability	☐	Knowledge of Youth's Strengths/ Needs
☐	Caregiver Optimism	☐	Family Stress	☐	Caregiver Resourcefulness
☐	Residential Stability	☐	Caregiver Physical/ Medical Needs	☐	Caregiver Mental Health Needs
☐	Caregiver Substance Use Needs	☐	Caregiver Needs related to Intellectual/ Developmental Disability	☐	Caregiver's ability to provide a Safe Environment
☐	Caregiver Needs involving Transitioning from Military Services				

Detailed description of all checked caregiver strengths and needs:

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For youth being referred for Out-of-Home Treatment:

If the youth has not been successful with community-based services and supports - please describe, in detail, the reasons why community based services and supports were not successful.
What are the resources, supports, and alternative interventions that could be considered in order to potentially maintain the youth in the community? Please comment on caregivers strengths, abilities, and limitations in regards to successful maintenance of the youth in the community:

Clinical Summary Template

History of Treatment & Youth System Involvement:

(Provide detailed information for all applicable sections including start/end dates and name of provider/agency/facility – limit to the past 3 years):

Behavioral Health (include: Outpatient, Intensive In-Home, Partial Hospitalization/ Partial Care Programs, Out of Home, Inpatient Hospitalizations)

Substance Use (include type of substance, pattern of use, age of onset, types of treatment- Outpatient, Intensive Outpatient, Partial Hospitalization Programs, Short Term Residential, Long Term Residential, Detoxification, Inpatient hospitalizations):

Physical Health/ Medical (include active issues):

Specialty Needs (include fire setting, problematic sexual behavior, cruelty to animals, and assaultive behaviors, which has occurred at any point in their lifetime – also include details in regards to the severity and frequency of these behaviors and timeline with most recent occurrence):

Youth Welfare/DCP&P (include history of abuse or neglect and descriptions including duration, perpetrator; services including in home services, resource/foster home, therapeutic treatment home):

Juvenile Justice (include case pending, FCIU, probation, detention, day program, DAP, incarceration, parole):

School (include type of school placement, i.e. regular education, special education, in district/out of district, home instruction):

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Current DSM-5 Behavioral Health Diagnoses – All Required:

Name of practitioner who diagnosed youth:	
Credentials:	Date of diagnosis:
List all Diagnosis:	
Current Intellectual/Developmental Disability Diagnosis, if any:	
Most Recent IQ, if known:	
Current Prescription Medications: (Specify all - Name, dosage, frequency, start and end dates):	

Clinical Summary/Formulation:

(Please describe in a brief statement what are the presenting clinical needs, the overall clinical impression and diagnosis, and recommended treatment plan or strategies including frequency, intensity, and duration of interventions):



Clinical Summary Template

Information Sources (e.g. Parents, Foster Parents, Group Home Worker, Probation Officer, teacher etc.):

Name	Relationship to Youth	Phone Number

Printed Name of Assessor:	
Signature of Assessor:	
Credentials:	Date:

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